

Turkana County Health Facility Access & Planning Priority Brief

Status: Draft for stakeholder validation **Prepared for:** Turkana County health planning and partner feedback
Prepared by: Kevin Kirui **Date:** June 2026 **Data source:** Kenya Health Facilities Dashboard CSV export and dashboard planning indicators **Population denominator:** 2019 county population figures used in the dashboard

Executive Summary

This brief translates Kenya health facility data into county-level planning intelligence for Turkana. It supports one core planning question:

Where should Turkana prioritize facility strengthening, service expansion, and partner support to improve public-sector access and reduce avoidable referral burden?

The dashboard lists **170 health facilities** in Turkana County. Of these, **145 are operational**, **15 are not operational**, and **10 are pending opening**. Public facilities account for the largest share of the county facility base, while faith-based providers form a significant support layer.

Turkana ranks **15th out of 47 counties** in the Planning Priority Index, with an overall score of **60.0**, placing it in the **medium-priority** category. The strongest planning signal is **high service risk**, with a service risk score of **80.0**.

This suggests that Turkana's planning challenge is not only the number of facilities. The bigger question is whether existing facilities provide the right services, capacity, and referral support across a large remote county.

Key findings

- 1. Turkana has moderate facility density but high service risk.** Turkana has **18.3 facilities per 100,000 people** and **10.8 public facilities per 100,000 people**, but the dashboard flags high service risk from low coverage.
 - 2. The facility network depends heavily on public and faith-based providers.** Public facilities account for **58.8%** of listed facilities, while faith-based facilities account for **24.1%**.
 - 3. Primary-care points dominate the facility network.** Dispensaries account for **116 of 170 facilities**, or **68.2%** of the facility base. Hospitals account for only **8 facilities**, or **4.7%**.
 - 4. Bed capacity is concentrated in a small number of facilities.** Turkana has **782 listed beds**. Hospitals account for **495 beds**, while health centres account for **265 beds**.
 - 5. The next validation step is service-level confirmation.** The current CSV export does not expose ART, FP, or IPD service columns. County and partner validation should confirm which services are missing, weak, inactive, or concentrated.
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1. Facility Access: Current State vs Peer Counties

1.1 Turkana Facility Access Baseline

Turkana County has **170 listed health facilities** in the dashboard. Of these, **100 are public facilities**, representing **58.8%** of the county facility base.

Private facilities account for **24 facilities**, or **14.1%**. Faith-based facilities account for **41 facilities**, or **24.1%**. The remaining facilities include **3 community facilities** and **2 NGO facilities**.

Using the 2019 population estimate of **926,976**, Turkana has approximately **18.3 health facilities per 100,000 people**. Public-sector facility density is approximately **10.8 public facilities per 100,000 people**.

This suggests that Turkana's health access story is not only about total facility count. The county also depends on a mixed delivery landscape where public facilities carry the largest share of access, while faith-based providers form a significant support layer.

1.2 Peer County Comparison

Metric	Turkana	Mandera	Marsabit	Nairobi
Population	926,976	867,457	459,785	4,397,073
Total facilities	170	87	115	941
Public facilities	100	56	73	146
Private facilities	24	30	26	558
Faith-based facilities	41	1	16	101
NGO facilities	2	0	0	119
Community / academic / other	3	0	0	17
All facilities per 100k	18.3	10.0	25.0	21.4
Public facilities per 100k	10.8	6.5	15.9	3.3
Private facilities per 100k	2.6	3.5	5.7	12.7

1.3 Ownership Share Comparison

Ownership share	Turkana	Mandera	Marsabit	Nairobi
Public share	58.8%	64.4%	63.5%	15.5%
Private share	14.1%	34.5%	22.6%	59.3%
Faith-based share	24.1%	1.1%	13.9%	10.7%
NGO share	1.2%	0.0%	0.0%	12.6%
Community / academic / other	1.8%	0.0%	0.0%	1.8%

1.4 Interpretation

Turkana County has **170 listed health facilities**, equivalent to approximately **18.3 facilities per 100,000 people**. This places Turkana above Mandera, which has **10.0 facilities per 100,000 people**, but below Marsabit, which has **25.0 facilities per 100,000 people**.

Nairobi has **21.4 facilities per 100,000 people**, but its facility landscape is structurally different because private providers dominate the county facility base.

Public-sector facility density gives a clearer planning signal. Turkana has approximately **10.8 public facilities per 100,000 people**, compared with **6.5 in Mandera**, **15.9 in Marsabit**, and **3.3 in Nairobi**.

This suggests that Turkana has a stronger public facility base than Mandera, but weaker public-sector density than Marsabit.

Ownership mix also matters. In Turkana, public facilities account for **58.8%** of listed facilities, while faith-based providers account for **24.1%**. This indicates that Turkana's facility access model depends heavily on public-sector delivery, supported by a significant faith-based provider layer.

For planning, Turkana should not be framed only as a low-facility county. The stronger issue is population-adjusted access pressure in a large remote county, where public and faith-based facilities carry much of the access burden.

2. Facility Functionality and Capacity

The Turkana facility dataset includes **170 listed health facilities**. Of these, **145 facilities are operational**, representing **85.3%** of the county facility base. Another **15 facilities are not operational**, while **10 facilities are pending opening**.

2.1 Operational Status

Status	Facilities	Share
Operational	145	85.3%
Not operational	15	8.8%
Pending opening	10	5.9%
Total	170	100.0%

2.2 Facility Category Mix

Facility category	Facilities	Share
Dispensary	116	68.2%
Clinic	28	16.5%
Health Centre	18	10.6%
Hospital	8	4.7%
Total	170	100.0%

The facility network is heavily weighted toward primary-care points. **Dispensaries account for 116 facilities, or 68.2% of all listed facilities.** Clinics account for **28 facilities**, health centres account for **18**, and hospitals account for **8**.

This means Turkana has broad primary-care presence through dispensaries, but higher-level care capacity appears concentrated in a small number of facilities.

2.3 Bed Capacity

Turkana has **782 listed beds** across the dataset.

Facility category	Beds
Hospital	495
Health Centre	265
Dispensary	17
Clinic	5
Total	782

Hospitals account for **495 of 782 beds**, while health centres account for **265 beds**. This suggests that inpatient and higher-level care capacity is concentrated in a small number of sites.

For planning, Turkana's access challenge should be assessed in two layers:

1. Whether communities can reach any facility.
2. Whether reachable facilities can provide the services and capacity needed to prevent avoidable referrals.

2.4 District Distribution

District	Facilities	Share
Turkana Central	48	28.2%
Turkana West	31	18.2%
Turkana North	27	15.9%
Turkana South	25	14.7%
Loima	24	14.1%
Turkana East	15	8.8%
Total	170	100.0%

2.5 Beds by District

District	Beds
Turkana Central	349

District	Beds
Turkana West	169
Turkana South	96
Loima	74
Turkana East	74
Turkana North	20
Total	782

Turkana Central holds the largest share of listed bed capacity, with **349 beds**. Turkana North has **27 facilities** but only **20 listed beds**, which should be validated with county teams.

This may indicate a planning concern: some districts may have facility presence but limited inpatient capacity.

3. Service Availability: Critical Gaps

The dashboard Planning Priority Index flags Turkana's strongest issue as **high service risk from low coverage score**.

Turkana's service risk score is **80.0**, higher than its access risk score of **71.9** and ownership risk score of **18.5**.

The current CSV export does not include service-specific columns for ART, FP, or IPD. Because of this, this draft does not claim exact ART, FP, or IPD availability counts.

Instead, this section should be treated as a validation prompt for county and partner review.

3.1 Service Validation Questions

County and partner teams should validate:

- Which facilities currently provide ART services?
- Which facilities provide family planning services?
- Which facilities provide inpatient services?
- Which facilities are listed as operational but not providing expected services?
- Which services are concentrated in a small number of facilities?
- Which service gaps create avoidable referral burden?
- Which gaps can be solved through service strengthening rather than new facility construction?
- Which gaps require staffing, commodities, equipment, or referral transport support?

3.2 Interpretation

Turkana's high service-risk score suggests that the county's planning challenge is not only about the number of facilities.

The key planning question is whether existing public, faith-based, and NGO-supported facilities provide the services needed to reduce referral pressure across a large remote county.

This makes service availability validation a priority before final recommendations are made.

4. Planning Priority Index Signal

Turkana ranks **15th out of 47 counties** in the Planning Priority Index, with an overall score of **60.0**, placing it in the **medium-priority** category.

Indicator	Turkana value
Rank	15 of 47
Overall score	60.0
Priority level	Medium
Access risk	71.9
Service risk	80.0
Ownership risk	18.5
Population factor	35.2
Main signal	High service risk from low coverage score

The main signal is **high service risk from low coverage score**. Turkana's service risk score is **80.0**, which is higher than its access risk score of **71.9** and far higher than its ownership risk score of **18.5**.

This suggests that Turkana's planning challenge is not only the number of facilities. The bigger issue is whether existing facilities provide the services needed to reduce referral burden and improve access to essential care.

For county planning, this shifts the brief toward three practical questions:

1. Which essential services are missing or weak across Turkana's facility network?
2. Which public or faith-based facilities could be strengthened fastest?
3. Which service gaps require partner support rather than new facility construction?

5. Planning Priorities

Priority 1: Validate service availability across operational facilities

Current signal: Turkana has a high service risk score of **80.0**, but the facility export does not show service-specific columns.

Recommended action: Validate ART, FP, IPD, maternal health, emergency referral, and inpatient service availability against county records.

Why this matters: A facility can be operational but still lack the services needed to reduce referral burden.

Suggested validation owners: County Health Records / M&E team, County Planning Officer, sub-county health teams, and partner program leads.

Priority 2: Strengthen service capacity in existing public and faith-based facilities

Current signal: Public facilities account for **58.8%** of facilities. Faith-based facilities account for **24.1%**. Together, they form the main access backbone in Turkana.

Recommended action: Identify existing public and faith-based facilities that can add or strengthen essential services without requiring new facility construction.

Why this matters: Service strengthening may produce faster planning value than building new facilities, especially where basic facility access already exists.

Priority 3: Review districts with low bed capacity relative to facility count

Current signal: Turkana North has **27 facilities** but only **20 listed beds**. Turkana Central has **48 facilities** and **349 beds**.

Recommended action: Validate whether bed distribution reflects current facility capacity, referral routes, staffing, and actual inpatient demand.

Why this matters: Districts may have many primary-care facilities but limited higher-level care capacity. This can increase referral burden and delay access to inpatient care.

Priority 4: Use partner support for service readiness gaps

Current signal: Turkana has a significant faith-based facility layer and a small NGO facility presence. Service-risk scoring suggests that coverage gaps may need targeted support.

Recommended action: Use partner discussions to focus on service readiness, referral support, equipment, staffing, commodities, and outreach models.

Why this matters: Partner support may be most useful where a facility exists but lacks the capacity to deliver priority services reliably.

6. How to Use This Brief

For County Annual Health Planning

Use this brief to:

- Identify where facility count differs from service capacity.
- Support discussion on public-sector strengthening.
- Compare Turkana against peer counties with similar access challenges.
- Guide facility and service validation before budget allocation.

For Donor and Partner Meetings

Use this brief to:

- Show where service-risk signals need partner validation.
- Discuss quick wins through existing public and faith-based facilities.
- Align partner support around service readiness rather than only infrastructure.

- Identify where referral burden may be reduced through targeted service expansion.

For County Health Management Team Review

Use this brief to:

- Validate facility functionality and service availability.
- Confirm whether non-operational and pending-opening facilities are still accurately listed.
- Identify which districts need deeper review.
- Decide which indicators should be updated quarterly.

7. Recommended Validation Questions

Use these questions in feedback calls or county review meetings:

1. Do the listed facility counts match Turkana County records?
2. Are the **145 operational facilities** still operational?
3. Are the **15 not-operational facilities** correctly classified?
4. Are the **10 pending-opening facilities** still pending, already open, or inactive?
5. Which facilities currently provide ART services?
6. Which facilities currently provide family planning services?
7. Which facilities currently provide inpatient services?
8. Which districts face the highest avoidable referral burden?
9. Which facilities could add services fastest with partner support?
10. Which service gaps are caused by staffing, equipment, commodities, referral transport, or infrastructure?

8. Data Notes and Transparency

Data used

This draft uses:

- Turkana facility CSV export from the Kenya Health Facilities Dashboard.
- Planning Priority Index values shown in the dashboard.
- County population denominators used in the dashboard.
- Peer comparison values manually captured from the dashboard.

Current data limitations

- The CSV export does not include ART, FP, or IPD service columns.
- Service availability must be validated against county records or a service-level dataset.
- Facility status may have changed since the source data was collected.
- Bed counts may require verification.
- Facility presence does not equal service readiness.
- Geographic access barriers such as road condition, distance, insecurity, flooding, and transport availability are not captured in this draft.
- Sub-county population denominators are not included in this draft.

How to interpret this draft

This brief should not be treated as a final planning decision document.

It is a validation draft designed to help county and partner teams answer:

- Does this data match current reality?
 - Which signals are useful for planning?
 - Which service gaps need deeper analysis?
 - What should be added before this becomes a final county planning brief?
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Appendix A: Peer County Selection Rationale

Mandera

Mandera provides a useful peer comparison for Turkana because both counties face remote-access and ASAL health planning challenges.

The dashboard lists **87 facilities** in Mandera, compared with **170 facilities** in Turkana. Using 2019 population figures, Mandera has approximately **10.0 facilities per 100,000 people**, compared with **18.3 facilities per 100,000 people** in Turkana.

Mandera also has lower public-sector facility density, with approximately **6.5 public facilities per 100,000 people**, compared with **10.8 public facilities per 100,000 people** in Turkana.

Its facility mix is more dependent on private providers, which account for **34.5%** of listed facilities, while faith-based ownership is minimal at **1.1%**.

This comparison suggests that Turkana's access challenge remains important, but Mandera may represent a more severe facility-density gap.

Marsabit

Marsabit provides a useful ASAL peer comparison because it has a smaller population but a relatively strong facility base.

The dashboard lists **115 health facilities** in Marsabit, including **73 public facilities**, **26 private facilities**, and **16 faith-based facilities**.

Using 2019 population figures, Marsabit has approximately **25.0 facilities per 100,000 people**, compared with **18.3 in Turkana** and **10.0 in Mandera**.

Marsabit also has stronger public-sector density, with approximately **15.9 public facilities per 100,000 people**, compared with **10.8 in Turkana**.

This suggests that Turkana's access challenge is not simply about low facility count. Compared with Marsabit, Turkana's larger population dilutes facility access, even though Turkana has more facilities in absolute terms.

Nairobi

Nairobi provides a useful contrast case because it shows a different type of planning challenge.

Nairobi has **941 listed facilities**, equal to approximately **21.4 facilities per 100,000 people**. But public facilities account for only **146 facilities**, or **15.5%** of Nairobi's facility base.

Private providers account for **558 facilities**, or **59.3%**.

This makes Nairobi useful as an ownership-imbalance comparison. Turkana's facility base is public and faith-based heavy, while Nairobi's is private-sector heavy.

The comparison helps show why facility count alone is not enough. Ownership mix matters because it affects affordability, public-sector planning, equity, and access for vulnerable populations.

Appendix B: Turkana District Detail

Facilities by District

District	Facilities	Share
Turkana Central	48	28.2%
Turkana West	31	18.2%
Turkana North	27	15.9%
Turkana South	25	14.7%
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Turkana East	15	8.8%
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Beds by District

District	Beds
Turkana Central	349
Turkana West	169
Turkana South	96
Loima	74
Turkana East	74
Turkana North	20
Total	782

Interpretation

Turkana Central has the largest number of facilities and the largest listed bed capacity. Turkana East has the fewest listed facilities. Turkana North has more facilities than Turkana East but far fewer listed beds.

These patterns should be validated with county teams before drawing final conclusions about referral burden, inpatient access, or district-level service gaps.

Appendix C: Next Data Needed

To improve the next version of this brief, the following data should be added:

- ART service availability by facility.
 - Family planning service availability by facility.
 - IPD or inpatient service availability by facility.
 - Maternal health service availability.
 - Emergency referral capacity.
 - Facility functionality status from current county records.
 - Staffing availability.
 - Commodity availability.
 - Sub-county population denominators.
 - Facility coordinates.
 - Travel-time or road access estimates.
 - Referral pathway data.
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Contact

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